Expansion of Chowgacha Jhenaidah (C-J) community involved health management model

Draft National Strategic Plan 2019-22





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Coordination and Support Centre DGHS Mohakhali Dhaka-1212

Expansion of Chowgacha Jhenaidah (C-J) community involved health management model

Introduction

Bangladesh is committed to implement Universal Health coverage (UHC) by 2032 like other countries of United Nations. But it may be a challenge to provide quality healthcare while securing economic security to all the people of the country. Many of the people do not get health service from government hospital, only 23% of the maternal deliveries are conducted in government hospital. Many of the Upazilla Health Complex is underutilized. To address these problems, necessary initiatives need to be taken in district and Upazilla level hospitals as well as hospitals at any level throughout the country, to reach the desired goal of performance. Standard improvement of the hospital management; ensuring transparency and responsibility of health providers; organizing health manpower in a team spirit; regular supply of manpower, logistics, equipment, medicine and other necessary health components; community and administration's engagement - without these components, implementation of universal health coverage is not possible.

Chowgacha Jhenaidah (C - J) community involved health management model is approved by the Bangladesh Government and World Health Organization. The main components of (C - J) model are target oriented activities, maximum utilization of government resources and support from the community, involvement of the community, ownership development. Many

of the hospitals like Chowgacha and Jhenaidah implemented community involved system, the managers having the quality of leadership, dedication for health service, to attend the hospital full time, well behaviour, transparent economic management, provide health service in a team, goal oriented health activities, maximum utilization of government resources, collecting the resources locally, if needed, to address manpower and equipment unavailability, and to maintain relationship with the local people and administration etc. These health facilities solve their problems easily by the support of the community. The other components of the model are, provision of manpower and supply of necessary health equipment with the help of local people and the private donors. Moreover, ensuring safety of the service providers (Doctors, Nurses and other health workers) and creating people-friendly workplace. In this health management model, local people's representative, local administration local physician, lawyers, journalists, police, teacher, NGO and industrialist and wealthy people of the community, students and representatives of different class-profession are involved. Where the model has been replicated partially or completely, their the local resources are mobilized and utilised those maximum and various types of corruption have been prevented and overall patient friendly environment have been created whereas healthcare quality have been improved exemplarily. ####

By taking above initiatives the performance of Chowgacha Health complex increased significantly. From a statistical data of Chowgacha health complex, it is shown that bed occupancy rate of Chowgacha UHC was 78.9% in 1997

which improved to 208% within 2011. First visit of antenatal care was 16.3% (817) in 1996 increase to 170% (8935) in 2011, hospital delivery rate was 2.1% (99) in 1996, to incise 98.4% (4413) in 2011, caesarean section rate was 1.02% (01) in 1996 which increased to 22.5% (1007) within 2011. Community engaged health management model has developed Chowgacha UHC's important health statistics significantly. In Chowgacha UHC maternal mortality rate was 599/100,000 live births in 1995 which reduced to 68/100,000 live births. Neonatal mortality rate and under 5 mortality rate was 42/1000 live birth and 78/1000 live birth respectively in 1995 to decrease to 18.2 and 23.6 in 2011 respectively. Though the performance of the health complex is declining in 2017 due to deferent causes but the latest performance is higher than any other Upazilla Health Complex of the country. In Jhenaidah Sadar Hospital after implementing the model the performance of emergency and outdoor of the hospital was 81,795 in 1998 which increased to 340,932 in 2017. In indoor admitted patient was 12973 in 1998 to 35569 in 2017. The bed occupancy rate was 126% in 1998 to 242% in 2017. Total hospital delivery was 1106 to 2997 in 2017. The hospital delivery in Jhenaidah sadar hospital was highest among all the Government Hospital of Khulna Division in 2017. In case of Hospital delivery, Chowgacha Health Complex stood first in 2011 and Jhenaidah Sadar Hospital stood first in 2017 among all the Government Hospital in Khulna Division.

Excepting above 2 hospitals many of the hospitals of Bangladesh by follow the Chowgacha, Jhenaidah model improve their quantity and quality of the services. Those countries of the world achieved universal have coverage, community engagement and involvement of local people was important component. By follow this is a model Government of Bangladesh try to implement the universal have coverage by replicating CJ model throughout the country.

Evolution of CJ model.

Community participation management is in which person, family and society themselves try to improve the health development of their own, and capable to develop it. CJ model is those model if it is applied Upazilla and district hospital of Bangladesh Government wish based on morality maximum utilization of government resources, community engagement and achieve the health related goal in time. In this model the people become confident to the government health system and develop their ownership.

In community based health service system the people participate in health development by 1. Planning implementation management and evaluation 2. Infrastructure development, Human resources, and other logistic support 3. Community awareness build up and involvement utilization of Government Hospital. Expanded programme on immunization (EPI) and community clinic activities of Bangladesh Government are the bright examples of community engagement in Bangladesh. Chowgacha-Jhenaidah health management model is now the milestone of community engagement in district and Upazilla

hospital. Nearly 20 years Chowgacha health complex best health complex of the country for its quality and quantities service. At the same time nearly last 7 years Jhenaidah District Hospital is on of the best hospital of the country for its performance & quality.

Historical background

Chowgacha Jhenaidah community engagement model primarily starts in 1985 in Upazilla Health Complex of Chowgacha under Jessore district. Then the Health and family planning officer of the Upazilla health complex was freedom fighter Dr. Humayun Mollah, A group of young doctor worked under him and improve the outdoor and field services of the Upazilla.

Among the doctors Md. Emdadul Haque was one of them. With limited resources by their innovative activities they solve their challenges and improve the health service system. In 1986 with the help of Upazilla parisad they expand their health complex from a tally shed to nearby public library and opens indoor of six beds. A EPI building was also constructed by Upazilla parisad for EPI program. With the help of local administration, clubs and community engagement starts eye camp, diabetic centre ANC corner and other activities. Within a short time the health complex became very popular. The health complex was awarded nationally for its best performance for EPI in 1989. Zero tolerance about corruption in health service was one of the important component of the health complex. The health complex develop community ownership.

Under the leadership Dr. Md. Emdadul Haque community engagement system expended in Lohagara Upazilla Health Complex, Narial in 1989-1995.He got the idea of preventive health when he was the student of NIPSOM in1990-1991session.He also got the chance to meet and work with renound Professor Muklechur Rahman in Narail. He improve the health and family planning service of the Upazilla. With the help of community leaders, local administration, teacher, social worker, reporters, and high officials who born in Lohagara combined with them made a Lohagara development plan in 1995 under the guidance of Prof. Muklechur Rahman and local administration.

Six bedded Chowgacha Upazilla Health Complex shifted to 31 bedded new health complex building in 1994. At that time upazilla health and family planning officer was freedom fighter Dr. Abdul Mannan. He also starts there six bedded maternity word in 1994 for basic emergency obstructed care. (EOC). Dr. Emdadul Haque again joined in Chowgacha Upazilla health complex in 1995 with the experience of community engagement. At that time they starts, health education corner, ORT Corner, EPI Corner, and birth and death data collection throughout the Upazilla was launched - which was not usually not done in the health complex. In 1995-1998, mother and child health development program was implemented in Chowgacha Health Complex with the help of JICA (Japan) under Frontline MCH Program. They were involved in logistics and technical support.

In August 1996, Dr. ASM Abdur Razzaq and Dr. Selina Khatun joined the Chowgacha Health Complex. At that time MCH program was under the directorate of family planning and the service was not satisfactory in the Upazilla. The local health authority took the initiatives to develop the MCH service which opened ANC corner for pregnant mother and under 5 corner for under 5 children. The outdoor service of patient is divided into male, female, under 5 baby and ANC mother. For every group of patient one doctor is appointed and responsible for the patient. Running blood bank system started in 16th December 1996 to meet up emergency safe blood transfusion. A base line survey report was made in January 1997. At that time local administration, industrialist and community played a role to improve sanitation and prevention of early marriage. Then all the 8 (eight) components of primary health care was effectively implemented in Chowgacha health complex which was one of the example in Bangladesh. Innovation and community engagement was the key point of successful. In 1998 eight (8) nurses of Chowgacha health complex was trained on EOC from Ad-din Hospital Jessore under frontline MCH program. At that time the nurses were not so experienced as today on management of normal delivery. With the help of UNICEF comprehensive emergency obstetric service started in Chowgacha UHC in 2000. In 2001 by local level plan a vision was develop for the development of maternal and child health, which was known as vision by the year 2010. The motto of this vision was every pregnant mother will get 5 antenatal visits directly from the health complex and every mother will deliver their baby in the health complex. This visionary target was an exceptional example in the country. In 2005 Chowgacha was awarded

nationally for its best performance in EOC services and continued until 2014. At that time Dr. Salahuddin Khan was the Upazilla health family planning officer. Community supported manpower supports the health complex from 1998. A new building with 41 maternity bed for EOC service started in 2006 with the help of community supported manpower. Nearly 46 manpower was supported by the community. Among them 26 were volunteers and 12 cleaners. In 2007, DSF program started for poor pregnant mothers which also helps in development of maternal health.

In 2008 the health complex was upgraded from 31 to 50 bedded hospital and the Ministry of Health and Family Welfare declared the health complex as the Model Upazilla Health Complex of Bangladesh. In 2009 Chowgacha Upazilla achieved health related millennium development goal 5. In 2010 Director General of WHO Dr. Margaret Cahn visited the Chowgacha Health Complex and recommended to replicate the model throughout the country.

In January 2011 Chowgacha health complex starts NCD prevention programme by local initiative, which was also a exceptional example in the country. In July 2011, 5's KAIZEN-TQM started in Chowgacha UHC for quality development by SMPP (JICA). In December 2011 in Chowgacha Upazilla health complex hospital delivery rate was more than 90% which was the outstanding performance among all the Upazilla health complex in Bangladesh. In 2018 Chowgacha Upazilla health complex stood first and

awarded by Health and Family Welfare Ministry. In May 2017 Government took the initiative of upgraded the hospital to 100 (hundred) bedded.

Dr. Md. Emdadul Haque joined in Jhenaidah Sadar Hospital as senior consultant (Obst & Gynae) in May 2012. Jhenaidah Sadar Hospital was upgraded from 50 to 100 bedded hospital in 1994 There was no post of aye-word boy, guard, gardener, trolley-man, and electrician. There were only 5 cleaners. About 11 unit were functioning 24 hours. For all the units needed 36 cleaners and 36 aye, word boy. But there were only 5 (five)workers. The hospital was in secured, un-cleaned and the environment was not suitable to serve the patient. 48 non-government workers work there. They would take fees from the patient. The performance and quality of this service was not so good. He took the initiative to develop the hospital as model deistic hospital by follow the community engaged Chowgacha health complex model. He arranged 56 community supported man power for the hospital. The post of human resource was sanctioned in 2014 and fill up in 2016. In 2014 Health Economic Unit (HEU) of Health and Family Welfare Ministry took the initiative of piloting as Model District Hospital. In 2015, QIS of health economic unit replicate this model in 10 District Hospital and 5 Upazilla health complex throughout the country. Dr. Aminul Hasan was the focal person in HEU for this model.

In 2007, Dr. Emdadul Haque worked in Monirampur Upazilla Health Complex for 6 months. He reformed the service system of the health complex. By local initiatives and starts ANC corner, under 5 corner, health education corner, comprehensive EOC service, improved cleanness and beautification of the hospitals, starting a safe blood transfusion system. The hospital was awarded for best performance in EOC. All the components of Chowgacha model was implemented in Monirampur within 6 months. The service system were established under the guidance of Civil Surgeon , RMO and medical officer Dr. Dilip Kumar Roy.

Kotchadpur Upazilla Health Complex

Characteristics (C-J) community engaged health management model:

1. Successful leadership in health management

2. Personal and group skills in health care

3. Community engaged health management

4. Effective relation with local administration and with other government organizations.

5. Improvement of security by public relations.

6. Improved maternal health care activities

7. Improved child health care activities

8. Ensuring corner-based health care

9. Birth and death recording, creating progress assessment and accountability by collecting information on all health indicators.

10. Health awareness develop by health education.

11. Organizational cleanliness (Waste management) and beautification.

12. Provision of safe blood transfusion for emergency needs.

13. Zero tolerance again st corruption.

14. Implementation of the 5S KAIZEN-TQM for the improvement of health care.

15.Improved financial management

15. Health care development by unit management

1. Successful leadership in health management

The role of leadership in the creation of the C- J model was unrivaled. Successful implementation of public health management has been possible in the successful application of honesty, sincerity, sympathy, empathy, etc. One or joint leadership played unique role in this model. plays as a role model both institutions. However, joint leadership, co-operation and coordination of same mentality colleagues played an important role in implementing this model. Upazilla health and family planning officer of the Chowgacha Health

Complex and his colleague played a unique role in this regard on the other hand in Jhenaidah single leader played main role.

The success of this model depends on the role of someone or a skilled team. Introductory role of the individual is an important condition for the implementation of the model.

2. Personal and group skills in health care.

Successful staging of a team effort in health care, personal and teamwork skills and performance of all types of employees, including doctors, nurses, are extremely important. A successful leader encourages the team to form, manage and achieve desired success. In Jhenaidah and Chowgacha, the active role and positive attitude of health care providers and organizations are contributing to the development of the model.

Their initiative has been encouraged by the concerned health and healthcare providers. There is also the continued cooperation and encouragement of the Ministry of Health and Family Welfare and Director General of Health Services.

3. Community participation in health management

Health is a wealth. Ensuring health is a right. The health (preventive, curative, promotive, rehabilitative and palliative) security and development is essential for development. Identification of health problems and priority

selection and local planning an implementation is the essential activity of the community. It can be solve by the community. Utilization rate of the Government Health service can be increased by the community participation. The can also help in build the confidence of Government Health Services. By expansion of Government Service mortality rate decreased which affects the health indicators. The people representative, political leaders and civil societies can play important role in community engagement. Nearly 46 human recourses and logistics were supported in Chowgacha by the community2011 and 56 human recourses and many logistics are supported by the community in Jhinaidah Sadar Hospital in 2018.

4. Participation of local administration and other government organizations.

Local public administration plays an important role in any implementation of any programme. Upazilla Nirbahi Officer and other officials of government office are responsible for implementation of Government programme. They work under the leadership of Chairman, Upazilla Parisad. Mayor of Pouroshova plays and important role in the development health of the municipality area. Apart from the support of resources, active participation, supervision, natural or man-made disaster prevention, security etc is supported by local administration and people representative. Inter-departmental coordination among government departments including public administration, police, education and social welfare is very important for the development of health system. Other government organizations, including the

public administration, played an important role in the development of CJ Model's health system.

5. Improvement of security by public relations.

In recent times, security in the hospital has become an important issue. Different types of dissatisfaction of the patient or their relatives have occurred, the isolated scattered, sporadic attacks happening on the hospital.

Even in district hospital due to lack of adequate security service providers are often confronted with safety. In C-J mode, the all-time security arrangements have been ensured in this ideal management. Its important aspects to utilize volunteers in safety. Due to the formation of the Community Support compete, the public relations policy has created a new confidence-building environment for doctors and patients. The safety of the doctors, and other service provider or patients have been improved. Confidence has been created in each other.

6. Improved maternal health care services

At least 5 times of all pregnant mothers would take antenatal care directly from the health complex, and hundreds of mothers are received care for the hospital. Every mother of the Upazilla will deliver their baby in the Upazilla health complex. More than 90% hospital delivery was the target of the health complex by 2010. As a result of repeated antenatal visits in the hospital, the relationship between the service provider and the mothers were developed. So confidence about the hospital's activities was developed. As a result,

pregnant mothers became enthusiastic and courageous in the institutional delivery. By providing emergency delivery services (EOC) with utmost efficiency and sincerity, the rate of normal delivery has increased and the caesarean section decreased. As a result, the rates of maternal mortality and infant mortality decreased substantially. One of the prerequisites for improved maternity care is the safe blood transfusion in emergency. Health authority encourages the local people to donate blood and arrange immediate blood collection and blood transfusion for urgent need. Emergency care services are available for emergency mothers (24/7). To ease the tasks, the problem of manpower has been solved by volunteer (nursing assistant) system supported by the community. Extra bed arrangements have been made for maternity. The environment was women friendly & maintaining and confidentiality. Since 1996, for the poor pregnant mothers coming to Chowgacha Health Complex, the Matir Bank'' programme started to save money in everyday for emergency purposes.

7. Improved child health care services

Under 5 corner initiative starts since 1996, for the children under five years at Chowgacha Health Complex. Which was officially introduced in the name of IMCI Corner in 2007. In under 5 corner growth charts are used for measuring the nutritional status of the children. From here, all the children are given a primary treatment, including idea about EPI, diarrhoea, breast feeding, malnutrition. All the children from here are referred to a fixed doctor. It is to be noted that the separate child ward was opened in the indoor of the health

complex since 1996. These activities of Chowgacha is running continuously till to day. Due to these services child mortality and malnutrition status decreased. Neonatal mortality rate is also decrease due to high institutional delivery.

8. Ensuring corner-based health care

Skill workforce has been developed to provide quality health care. In the health complex, the health care service was divided into different units. In this model health institutions, on the basis of gender, age and health problems different corners have open such as for men, women, , IMCI, ANC PNC, VIA, EPI, breast feeding, ORT, Health Education, NCD etc. In the Upazilla health complex the indoor divided in to four categories-men, women, children and EOC departments. It develops the responsibility of the service providers. Due to the availability of Corner-Based Healthcare in the Outdoor, patients have been significantly reduced waiting time; overall, there has been improvement in disease-based health care services. These increased the number and satisfaction of the patient. While increasing the number of big operations due to ensuring the health services of the indoor department increases the number of beds also increased.

9. Birth and death recording, creating progress assessment and development of accountability.

To assess the progress of health and family planning activities, it is very important to collect birth and death information through the Upazilla. Even though birth data is collected in two of the department, weakness in collection of data is seen. As a result, it is not possible to know the exact birth and death data , birth rate, population growth rate, neonatal death rate, infant mortality rate, maternal mortality, and age & disease specific mortality rate. As a result, the responsibility of evaluating the progress of the program and making decisions accordingly, even the liability could not be created. This information was collected primarily since 1995 in Chowgacha Upazilla. Chowgacha Upazilla Health Complex has shown tremendous success in this year, 22 years since 1996.

10. Health awareness develop by health education.

Health education programs are vital in health institutions. Various national activities related to change in the behaviour of patients, such as maternity services, child health services, NCD, family planning, EPI, diarrhoea, pneumonia, rabbis etc. and to encourage the people to inform and encourage to utilize the government services. One of the main goal of health education is responsible officials will get information about the problems of the patients and take necessary action to solve it.

There are two such health education centres in Jhinaidah Sadar Hospital and three at Chowgacha Upazilla Health Complex. Each day, the program was operated through audio visual method. Health care centres have patient seats and other facilities. Birganj Upazilla Health Complex, which has public health facilities, has opened the library for patients and relatives. The officials of the organization have been associated with the highest level of staff at all levels. The program was conducted by distributing and distributing responsibilities through weekly roster.

11. Organizational cleanliness and beautification.

The cleanliness of the health organization keeps the disease away from the danger of infection in the hospital, on the other hand the odourless environment helps keep the sensitive minds of the patients happy. Cleanliness is not only in the hospital, it is essential to keep the entire area clean and clean. Organized flower gardens, medicinal gardens, nutrition gardens, and regular service are arranged in the public hospital with public health management. Cleanliness and beauty have significantly improved in relation to the over-exorbitant manpower maintenance work, with the help of people's financial assistance due to the lack of public manpower. Due to the development of waste system, the hospital has become clean and odourless. Healthcare providers, including physicians, formally participate in cleanliness of the entire hospital and premises at any national day on any national day. This is called "Big Cleansing Day".

12. Provision of safe blood transfusion for emergency needs.

Safe Blood transfusion is an important component of health care during the operation of any major surgery, especially during the Caesarean operation, APH and PPH. Blood grouping activities were started in the Chowgacha Health Complex to collect blood donor on the Victory Day of 1996. Those people are listed by blood grouping, whose residence within 1.5 kilometre of the health complex. About 600 blood donors were listed. These blood donors are called "Moving blood banks". Since then there have never been a shortage of blood for the patients of Chowgacha Health Complex. As a result, maternal lives have shaved due to emergency blood transfusion. Special efforts have been made for each National Day to collect blood donor. In Durga Puja mondops taken the initiative to collect blood donor & achieve success.

Another responsible group was formed to collect blood from the blood donors at night. In 2005, safe blood transfusion unit was established in collaboration with UNICEF for safe blood transfusion at Chowgacha Health Complex.

13. Zero tolerance against corruption.

One of the problems of the hospital is irregularities and corruption. People are providing help to doctors and health service provider to tackle such problems. Due to the role of the Community Support committee, some of the influential people could not get any illegal benefit from the hospital. So

revenue income have been increased. It is also easier to stop the misappropriation from patients in the name of Sontusti It has been possible to free the patients and the organization from the hostile condition of a class extremist named as a helper. In case of issuing medical certificate, tackling of illegal pressure, the corruption of the 'brokers' has been successfully prevented

14. Implementation of the 5S KAIZEN-TQM for the improvement of health care.

To maintain a quality health care services by available govt. resources to follow the philosophy of 5S KAIZEN-TQM

The first step of quality service in the hospital are 5S.

1st Sort - Removing unnecessary accessories from workplace.

2nd Set- Keep the necessary accessories properly

3rd Shine - Maintain cleanliness

4th Standardized - ensure the quality of the above three works

5th Sustain - Continuation of the mentioned activities.

Kaizen the Japanese word Kaizen means the means for a change or problem-solving process.

It is possible to make quality service using the 5S KAIZEN-TQM.

15. Health care development by unit management

One of the key features of this model is to ensure the overall service of the patients considering the patient's needs without considering the health services partly . In this management the service provider does not think that his responsibility is complete only after completing his own professional work. The next step is to follow whether the patient is getting the necessary services. Head of a ward considers all aspects of service management until his patients are leave from the hospital. The timely attendance of the officers and employees of his unit, cleanliness of the unit, food supply of patients, control of the ward, and all the issues including control of the crowd. These issues has been given importance in Jhenaidah Hospital. The health care providers at the same time interacted with their special work as well as management aspects of creative and human behaviour.

The rationale for the expansion of the Chowgacha-Jhenaidah (C-J) model

After the successful implementation of the Millennium Development Goals, by 2032, the progress of the universal health coverage and sustainable development goal has been going forward. It is not possible for the government to implement this huge activity, as there are several limitations. In order to implement this program, the involvement of the community with the government is essential. Chowgacha-Jhenaidah (C-J) model with community health engaged management has already established an example of health service. In the last 22 years, this model has been successfully going

on at Chowgacha Upazilla Health Complex in Jessore and in Jhenaidah Sadar Hospital for last 6 years. The program is also implementing in 15 district and Upazilla hospital of the country.

. In order to achieve sustainable development goal and universal health coverage in Bangladesh with the during the scheduled time, the Chowgacha-Jhenaidah (C-J) community engagement health management model will be expanded and implemented in the Upazilla-district level throughout the country, based on evidence-based, time-consuming and logical initiatives. It is to be noted that in 2010,Director General of World Health Organization visited to Chowgacha Upazilla Health Complex and recommended to replicate the model throughout the country. There is also a order to replicate C-J Model nationwide from the Hon'ble Prime Minister's Office.

Goal

Replication of C-J Model within 2022 at Upazilla Health Complex and District Sadar Hospital in Bangladesh

Expansion target

By June 2020 - 40 Upazilla and 20 district hospital;

By June 2021 - more than 250 Upazilla and 35 districts hospital;

By the end of June 2022 - all the remaining Upazilla and district hospital.

Objective

1. Take steps at the local level to achieve the goal of universal health protection for health development goals and sustainable development targets of the 7th Five Year Plan of Bangladesh Government.

2. Assets and opportunities allocated at the government level - Ensuring maximum utilization of benefits.

3. Local people's participation in local health management and opportunities for multifaceted contributions.

4. Leadership development and motivation.

5. Improve health care through subject-related services.

6. Through the development of various committees, the government's health care plans, implementation, management and monitoring at local level.

7. Maintain consistent success, development and stability through regular monitoring and evaluation.

Strategies

1. Initially round-the-clock evaluation of all the Upazilla Health Complex and District Sadar Hospital at Chowgacha-Jhinaidah (C-J).

2. Elections for the implementation of the public health system and for the implementation of stage-based implementation of Upazilla and district hospitals.

3. Preparation of planning in national level for the expansion of this model and the different level committees.

4. Chowgacha - Jhenaidah (C-J) nationwide, to promote publicity and social movement in expanding the public health model.

5. Leadership development and management development training for selected Upazilla health and pop officers and practitioners of the district hospitals.

6. Upazilla Health and Pop Officials and District Hospital's necessary manpower commanding.

7. Accepting arrangements for supply of necessary supplies in selected Upazilla health and pop officers and district hospitals.

8. Assets and opportunities allocated in the government level - ensuring maximum utilization and deficit detection

9. Creating opportunities for people's participation and multifaceted contributions in health management at local level

10. In the grouping system, the committee has been working at the local level to implement public health system

11. Maintain institutional successive success, development and stability through the implementation and management of the government's plans at local level and regular monitoring and evaluation

Chowgacha Jhenaidah (C-J) Community Engaged Health Management Model extension committees

1. **(C-J) Community Engaged Health Management National Steering Committee**
2. **Chowgacha Jhenaidah (C-J) Community Engaged Health Management Model National Technical Committee**
3. **Chowgacha Jhenaidah (C-J) Community Engaged Health Management Model Community Support Committee in District Hospital**
4. **Chowgacha Jhenaidah (C-J) Community Engaged Health Management model in District Hospital Implementation committee**
5. **Chowgacha Jhenaidah(C-J)Community Engaged Health Management Community Support Committee in Upazilla UHC.**
6. **Chowgacha Jhenaidah (C-J) Community Engaged Health Management implementation committee** in Upazilla UHC

CJ Model Community Engaged Health Service National Steering Committee

Serial	Designation	Position in Committee
1.	Secretary, Health service division, MOHFPW	Chair Person
2.	Additional directorate general(Administration), DGHS	Member Secretary
3.	Directorate General, DGHS	Member
4.	Directorate General, Family planning, DGFP	Member
5.	Directorate General (Nursing)	Member
6.	Additional secretary (Public Health), Health service division, MOHFPW	Member
7.	Additional secretary (Hospital) Health service division, MOHFPW	Member
8.	Additional secretary, (Budget, Evaluation and Activities), Ministry of social welfare	Member
9.	Directorate General (HEU), MOH & FW	Member
10.	Directorate General (Admin), Ministry of Local Govt.	Member
11.	Line Director (HMS) & Director (Hospital) DGHS.	
12.	Member Secretary CJ community engaged health management technical committee	Member

Responsibilities

1. Approve policy and strategic plan
2. Approve of the recommendations of CJ Model national technical committee.
3. Approved and allocation of budget.
4. Two meeting should be done in every year.
5. New member should be co-opted according to need

C-J Model Community Engaged Health service national technical committee

Serial	Designation	Position in Committee
1.	Director General of Health Services , DGHS	Advisor
2.	Additional Directorate general (Administration)	Chairperson
3.	Chief Coordinator, Coordination and support centre, DGHS	Member Secretary
4.	Director (Administration), DGHS	Member
5.	Director (Admin) DG Nursing	Members
6.	Director (Finance), DGHS	Member
7.	Director (Hospital), DGHS	Member
8.	Director (Primary Health Care), DGHS	Member
9.	Line Director, Community Based Health Care, DGHS	Member
10.	Assistant Director (Coordination), DGHS	Member
11.	Dr. Aminul Hasan, Focal Person, QIS Secretary, Health Economy Unit, Ministry of Health and Family Welfare	Member
12.	Dr. Iqbal Anwar, Senior Scientist, Health System and population studies division, ICDDR,B	Member
13.	Representative, UNICEF, Dhaka	Member
14.	Representative, WHO, Dhaka, Bangladesh	Member
15.	Representative, JICA, Dhaka	Member
16.	Representative of Bangladesh Medical Association (BMA)	Member
17.	Dr. Mushtuq Hossain, Coordinator, Coordination and Support Centre, DGHS	Member
18.	Dr. Md.Emdadul Haque, Senior Consultant Gynae, Jhenaidah Sadar Hospital [PRL]	Member and Consultant
19.	Medical Officer (3 Personals), , Coordinator, Coordination and Support Centre, Health Dept.	Member

Responsibilities:

1. Replication of Chowgacha Jhenaidah community engaged health management model.
2. Preparation of action plan, , order, SOP. APS auditing form related to the model
3. Evaluation analysis and preparation of summary note ordered by the ministry of health and family welfare
4. Preparation of training manual
5. Organize training
6. Play role in improving the quality of Upazilla and district hospital
7. Follow the order of DGHS related to the model.
8. The meeting of the will be held minimum one time per month, or more if necessary
9. The committee could co-opt if needed

3.C-J Model Community engaged district Sadar **1.**	Mayor, Municipality of district town/ Zillah Parisad Chairman	**Hospital Committee.** **SL NO** **Designation** Chairperson
2.	Super .district hospital	Member Secretary
3.	Civil surgeon	Co ordinator
4.	Representative of honourable MP	Member
5.	Representative of Zilla parishad Chairman	Member
6	Representative of DC	Member
7.	Representative of SP	Member
8.	Representative of BMA	Member
9.	Representative of BAR	Member
10.	RMO	Member/Focal Person
11.	Representative of Consultant	Member
12	Representative of Press Club	Member
13	Representative of BAR	Member
14.	Representative of NGO	Member
15.	Representative of Civil Society	Member
16..	Representative of DD Social welfare off	Member

Responsibilities:

1.Colliction of information about C-J model prepare a action plan to implement it.

2. To assist the hospital authorities in improving the quality of care, healthy environment, cleaning & security.

3.To assist the hospital authorities in optimum utilization govt. resources and its maintenance

4.To fulfil the demand & development of the hospital in access to govt. resources create a community support fund so that donors, organization can donate in the fund.

5.Identify the problems of health services and play a role to solve the problems locally

6.Encorrage the doctors, nurses and other service providers in hospital services and giving constrictive suggestion.

7.The committee should perform a meeting in every month and increase or decrease members also could made subcommittee according to the need.

4.C-J Model Implementation Committee in district Sadar Hospital

SL NO	Designation of Officer / Public Representative	Rank in committee
1.	Super, District Sadar Hospital	Chairperson
2.	Residential Medical Officer	Member Secretary
3.	Different WIT facilitator and team leaders	Member

Responsibilities:

1. Review the progress of the different WIT
2. Make an action plan for the next month.
3. Report to the higher authority in every month in a prescribed format.
4. Request to the community support committee in the form of proposals by determining which assistance is required from the community support committee.
5. Approval of the way of solution of different problems in WIT
6. The meeting of the committee should be held once in a month

5.C-J Model Upazilla Health Complex Community support Committee.

SL NO	Officer's Name	Rank in Committee
1	Mayor, Municipality/Upazilla Chairman	chair person
2.	Upazilla Health and Family Planning Officer	Member Secretary
3.	Representative of Upazilla Chairman(1)	Member
4.	Representative of the Honourable M P (1)	Member
5.	Representative of UNO (1)	Member
6.	Representatives of the union council's chairmen (1)	Member
7.	Representative of OC (1)	Member
8.	Upazilla Social Welfare Officer (1)	Member
9.	Representative of Press club (1)	Member
10.	Representative of NGO (UNO nominated) (1)	Member
11.	Representative of Civil society (UNO nominated)(1)	Member
12.	Representative of Consultant (1)	Member
13.	Residential Medical Officer (1)	Member

Responsibilities:

1.Colliction of information about C-J model prepare a action plan to implement it.

2. To assist the hospital authorities in improving the quality of care, healthy environment, cleaning & security.

3.To assist the hospital authorities in optimum utilization govt. resources and its maintenance

4.To fulfil the demand & development of the hospital in access to govt. resources create a community support fund so that donors, organization can donate in the fund.

5.Identify the problems of health services and play a role to solve the problems locally

6.Encorrage the doctors, nurses and other service providers in hospital services and giving constrictive suggestion.

7.The committee should perform a meeting in every month and increase or decrease members also could made subcommittee according to the need.

6.C-J Model Implementation Committee in Upazilla Health Complex

SL NO	**Designation**	**Rank in Committee**
1	**UH&FPO**	Chairperson
2	**RMO**	Member Secretary and Focal Person
3	Team Leaders and Facilitators of WIT Committees	Member

Responsibilities:

1. Review the progress of the different WIT
2. Make an action plan for the next month.
3. Report to the higher authority in every month in a prescribed format.
4. Request to the community support committee in the form of proposals by determining which assistance is required from the community support committee.
5. Approval of the way of solution of different problems in WIT

   The meeting of the committee should be held once in a month

Action plan of National Strategy for Implementation of C-J model

No	Work	Implementer	Duration	Comment
1.	Formation of national Technical Committee	DGHS	-	Completed
2.	Formation of the National Steering Committee	MOH&FW	December 18	-
3.	Formation of expert groups for implementation	National Technical Committee	December 18	-
4.	Primary survey nationwide	National Technical Committee, Coordination and support Centre, DGHS	-	Completed
5.	Visit of community supported health institutions	National Technical Committee		Completed
6.	Preparation of strategies	National Technical Committee, Coordination and Support centre (DGHS)	-	Running
7.	Finalization of strategy	National Steering Committee	December' 18	-
8.	First phase of district-Upazilla UHC identification	National Technical Committee	-	Completed

9.	collecting and analysing primary data of district and Upazilla hospital in the first phase,	National Technical Committee	December' 18-January' 19	Running
10	Prepare the Training Manual	National Technical Committee	December' 18-January' 19	-
11	Motivational program start in Field level by all stakeholders.	National Technical Committee, Director (CBHC)	January' 19	-
12	Starting for Implementation of C-J Model in first phase, district and Upazilla Hospital	Coordination and Support Centre DGHS, Director CBHC	January' 19	-
13	Formation of Community Support Groups	Civil Surgeon and superintendent of District Hospital of the concerned district	January' 19	-
14	Starting of second phase activities	Department of Health Coordination and Assistance centre, CBHC	March' 19	-
15	Sustainability of Chowgacha UHC and Jhinaidah Sadar Hospital	National Technical Committee and Community Support Committee	March' 19	Running

Institutional implementation Action plan

No	Program	Responsibilities	Duration
1.	Formation of Community Support Committee	Local Hospital / Upazilla Health Complex Authority	15 Day
2.	Preparation of initial assessment report	QIC and WIT team	30 Day
3.	Identification of Problems	QIC and WIT team	30 Day
4.	Determining the vision, goals and goals	QIC team	15 Day
5.	Motivation through the workshop to achieve the commitment of the people and the service providers to implement the goals	Local Hospital / Upazilla Health Complex Authority	90 Day
6.	Preparation and collection of necessary materials	Local Hospital / Upazilla Health Complex Authority	180 Day
7.	Prepare policies for building value-based work-environment an institutional culture	QIC Team	15 Day

8.	Communication with all stakeholders to develop social movements to achieve the goal	Community Support Group and Local Health Authority	60 Day
9.	Training on quality development 5S KAIZEN-TQM.	Local Health Authority	7 Day
10.	Achieve practical knowledge through a team visiting the Chowgacha UHC and Jhinaidah Sadar Hospital	National Technical Committee	3 days
11.	An expert team of the model would visit the hospital and develop them by hands-on	National Technical Committee	7 Day
12.	Evaluate the progress of the program	Local health authority	Every 90 days intervals

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Conclusion

Healthcare is a multidimensional humanized system. A huge task regarding of preventive, curative, promotive, rehabilitative &palliative health of the people. It is not possible to implement by government alone. So community engagement is necessary .Chowgacha-Jhenaidah model is a brilliant example of public health management. It has been possible to achieve public confidence by launching this management. The security of the service providers, the cleanliness& beatification of the environment and the quality of healthcare services have increased ie increase patient satisfaction rate. In this system people have the opportunity to be involved in health care, planning and implementation. As a result of public relations, it will be easier to identify health issues and to help solve problems in the future, in the light of their real experiences. This model will play a significant role in the implementation of the \"universal health protection\" program announced by the World Health Organization in Bangladesh.

Attachment

Table 1

Milestone of the development of the Chowgacha model.

1985	UHC extends to nearby public library and opens six bedded indoor with the help of Upazilla parisad
1989	National award for success in the EPI program
1994	The UHC shifted to 31-bedded new Upazilla Health Complex building .
1995	Starting the ORT Corner and starting of Frontline MCH program, and with the help of JICA.
1996	ANC corner, and under five corner starts.
1998	In January,1997 a baseline Survey was done and made a report.
2000	Comprehensive EOC program starts with the help of UNICEF.
2001	A plan for a vision by the year 2010 for achieving MCH service development .
2004-2014	Best Performance Award for EOC Services
2006	Safe Blood Transfusion Unit was opened with the help of UNICEF.
2007	Introducing of DSF program for poor pregnant mother& starting of Women Friendly Hospital initiative.
2008	Recognized as a Women Friendly Hospital

2008 June	Upgraded to 50 bedded and declared as a Model UHC by the MOH&FW.
2009	Achieving MDG 5 and 6
2010	Director General of WHO visited to Chowgacha UHC and recommends for replication .
2011 1st January	The successful targets of the target of 2010 and the launch of prevention of non-communicable disease(NCD) programs
2011 July	5S KAIZEN-TQM program starts by JAICA.
2017	Awarded to Health Minister's Award as stood 1st and also stood prize for EOC
2018	MOH&FW completed the plan for 100 bedded hospital

Table 2

Milestone of the activities of the Jhenaidah Model.

2012	Starting of Model activities.
2013	Kaizen Role Model Award for Model .
2014	Jhenaidah team visits Satkhira Sadar Hospital, Awarded second prize for TQM
2015	Ministry initiates Piloting of District-level Modelling and achieving the best prize for TQM.
2016	Replication of Jhenaidah Model in 10 district hospital&5 Upazilla UHC. Awarded, Health Ministers Award and Best performance Award for EOC.
2017	Awarded, Health Ministers Award and Best performance Award for EOC.

Table 3. Chowgacha Health Complex – improvement evaluation (1995-2017)

Subject	1995	1996	1997	2000	2003	2007	2011	2017
Bed Occupancy Rate %	-	-	78.9	93.5	88.9	201	208	171
ANC (First Visit)	-	16.3 (817)	24.6 (1228)	59 (2861)	88 (4484)	132 (7292)	170 (8935)	130 (6632)
Hospital Delivery Rate %	-	2.1 (99)	3.4 (151)	6.4 (422)	15 (661)	38.1 (1899)	98.4 (4413)	66 (2932)
C-Section rate %	-	.02 (1)	0	0.07 (3)	0.8 (36)	9.6 (473)	22.5 (1107)	17
Population Increase rate	1.87	1.86	1.66	1.7	1.57	-	1.36	1.17
Maternal Mortality Rate/100,000 live birth	599	300	206	168	114	160	68	91
Neonatal mortality rate/ 1000 live birth	42	47	42.4	23	28	21.5	18.2	16.6
Infant mortality rate/ 1000 live birth	65	58	56	36	34	29.2	23.6	18.6
Under 5 mortality rate/ 1000 live birth	78	67	62	44	43	33.3	25.7	21.1
Low birth weight rate(%)	-	-	36.5 (74)	15.2 (288)	13.7 (636)	-	-	-

Ref. UH&FPO, Chowgacha, Jessore

Table 4. Jhenaidah Sadar Hospital- Improvement Evaluation (1998-2017)

Serial	Subjecct	1998	2011	2012	2015	2017
1	Emergency & Out Door patient number	81795	170854	171070	277301	340932
2	Indoor patients	12973	22435	22951	29562	35569
3	Bed occupancy rate%	126	146	143	193	242
4	Number of EmOC service	256	1354	2268	13251	15360
5	Number of deliveries	1106	2375	2379	3034	2997
6	Operation (Major)	-	780	977	1582	1578
7	**Revenue income**	-	-	-	60,44,715	70,59,560

Ref. Superintendent, Sadar Hospital, Jhenaidah,

Table 5. Comparative Data of best performing EOC center of Khulna Division in 2017

Serial NO	Institution	Type of EOC Service					
		Normal		Caesarean		Total	
		2011	2017	2011	2017	2011	2017
1	Chowgacha Health Complex	3406	2163	1007	759	4413 (First)	2922 (Third)
2	Khulna Medical College Hospital	1657	1448	1356	1365	3013 (Second)	2524
3	250 Bedded Jessore General Hospital	1354	1172	1498	1778	2852 (Third)	2950 (Second)
4	250 Bedded Kashia General Hospital	1762	1595	915	919	2677	2514
5	100 Bedded Jhenaidah Sadar Hospital	1947	1854	430	1143	2337	2997 (First)

Ref. MIS,DGHS

Table 6. Comparative data of different countries resources utilized by the Govt.

SL NO	Country	Per capita health Expenditure per year by Govt. ($)	Doctors per 10000 Population	Nurse per 10000 Population	Beds per 10000 population	Out of pocket expenditure %
01	USA	9536	25.5	99	37	11.08
02	UK	4356	28.0	84	30	14.79
03	Malaysia	386	12.8	34	18	36.67
04	Sri Lanka	118	7.3	17.5	31	38.43
05	India	63	7.3	20.4	09	65.06
06	Nepal	44	08	4.7	-	60.41
07	Pakistan	38	08	06	06	66.40
08	Bangladesh	37	3.9	03	03	67.00

Ref: www.worldbank.org Accessed on 20/10/2018

Table.7. 7th Five Year Plan, Health, Nutrition and Population Target

SL No.	Indicator(effect/ results)	Base Year information	The target of 7th Five-Year Plan by 2022	SDG by 2030
1	Expected life expectancy (yearly)	70.4 (SVRS 2013)	72	
2	Under 5 mortality rate (per 1000 live births)	46 (BDHS 2014)	37	25
3	Infant mortality rate (per 1000 live births)	38 (BDHS 2014)	20	
4	Neonatal death rate	25 (2014)	18	12
5	Maternal mortality ratio (per 100,000 live births)	170 (MMEIG 2013)	105	70
6	Percentage of malnutrition among the under 5 children	32.6 (BDHS 2014)	20	
7	Percentage of stunting among the under 5 children	36.1 (BDHS 2014)	25	
8	Total fertility rate (children per head of every woman)	2.3 (BDHS 2014)	2.0	
9	Contraceptive prevalence rate (%)	62.4 (BDHS 2014)	75	
10	Immunization coverage under 1 year children	78 (BDHS)	95	
11	Percentage of childbirth by trained person	42.1 (BDHS 2014)	65	
12	Ratio of institutional delivery between poor and rich mother	15:69.5 (BDHS 2014)	1:3.5	
13	Tuberculosis identification rate (%)	53 (GTR 2014)	75	

14	Percentage of Antenatal Care (ANC) 4th visit	31.2	50	
15	Exclusive breastfeeding among under 6 months' baby	55.3 (2014)		
16	Adolescent fertility rate (15-19) per 1000 cuple	31	25	
17	Deaths by NCD per 10,000 population (30 - 69 years)			
18.a	Heart disease			
18.b	Cancer			
18.c	Diabetes			
18.d	Chronic respiratory disease			
19	Mortality in road traffic accidents (per 100,000 population)			
19.a	Male			
19.b	Women			
20.	Smoker's rate per 100,000 population (15+ years)			

Table. 8 Health related SDG goals, target (13), and Indicator (27)

SL No.	Target	Indicator	Index (effect / results)	Information Year Information Year (Start Year)	By 2030
1	3.1	3.1.1	Maternal mortality rate (per 100,000 live births)	170 (MMEI 2013)	70
2		3.1.2	Percentage of Delivery by physicians and trained person	42.1 (BDHS 2014)	
3	3.2	3.2.1	Under 5 Mortality rate / 1000 live births	46 (2014)	25
4		3.2.2	Neonatal death rate/1000 live births	25(2014)	12
5	3.3	3.3.1	HIV/AIDS incidence rate	0.01%	
6		3.3.2	TB Case detection rate	53 %(2014)	100%
7		3.3.3	Malaria incidence rate(High endemic area) 3 district	1-10/10,000pop	No public health problem
8		3.3.4	Hepatitis B incidence rate		
9	3.4	3.4.1	Death due to NCD(30-69)/10,000	18(WHO-2014)	6
		1	Heart Disease		
		2	Cancer		
		3	Diabetes		
		4	Ch .Resp disease		
10		3.4.2	Incidence of suicide/100 ,000 pop	8(2014)	2.67
		1	Male		

		2	Female		
11	3.5	3.5.1	Incidence of drug addiction/10,000pop		
12	3.6	3.6.1	Mortality rate due to RTA/100,000pop	85.6(World Bank-2002)	½ in 2020
			Male		
			Female		
13	3.7	3.7.1	CPR	62.4(2014)	
14		3.7.2	Fertility rate(15-19)/1000 couple	31	
15	3.8		Universal health Coverage/1000 pop		
16	3.9	3.9.1	Death rate due to water,air,soil pollution/10,000 pop		
17	3a		Tabaco use rate/1000pop(15+yr)	25(WHO2014)	
			Male	48	
			Female	02	
18	3b	3b1	Essential drugs for all	66%(2014)	100%
		3b2	Vaccine for all		
		3c3	Family planning commodities supply	84.4(2014)	
19	3.c	3.c	Health Manpower/100,000 pop		
		1	MO		
		2	Consultant(Clinical)		
		3	Consultant(preventive)		
		4	Nurse		
		5	Midwife		
		6	Public health nurse		
		7	Sanitary inspector		
20	3.d	3.d	Capabilities of disaster		

			management		
		3.d.1	Forecasting Capacities of disaster		
		3.d.2	Capacities of risk decreasing		
		3.d.3	Management development capacities build up		